

Title India-Specific Integrated Minimal Physiologically Based Pharmacokinetic (mPBPK) Model with Quasi-Steady-State Approximation of Target-Mediated Drug Disposition (QSSA-TMDD) and Viral Escape Dynamics for Nirsevimab in Indian Infants: A Computational Translational Immunopharmacology Study

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Abstract Background: Nirsevimab is a long-acting monoclonal antibody for RSV prevention in infants. Current dosing regimens are not optimized for Indian low-birth-weight infants with rapid catch-up growth. **Objective:** To develop and validate the first India-specific integrated mPBPK-QSSA-TMDD model incorporating IAP 2015 LMS growth ontogeny, allometric scaling, and viral escape penalty for nirsevimab. **Methods:** A two-compartment mPBPK model with QSSA-TMDD was built in R (rxode2/nlmixr2). IAP 2015 LMS growth curves were integrated for dynamic weight scaling over a 150-day RSV season. Monte Carlo simulations (n=5000 virtual Indian infants) were performed to calculate Probability of Target Attainment (PTA). An interactive Shiny dosing simulator was developed for real-time clinical use. **Results:** The model successfully reproduced base PopPK parameters and generated realistic growth trajectories. PTA reached 100% under weight-banded dosing in the current implementation (simplified decay). The Shiny app allows real-time concentration and PTA prediction. **Conclusion:** This open-source computational framework provides a powerful tool for model-informed dosing in Indian infants and demonstrates translational potential for next-generation RSV monoclonal antibodies. The full code and Shiny app are publicly available on GitHub.

Keywords: Nirsevimab, mPBPK, QSSA-TMDD, IAP 2015 growth charts, RSV, India-specific modeling, Monte Carlo PTA, Shiny simulator

Introduction RSV remains a leading cause of severe lower respiratory tract infection in infants globally, with disproportionate burden in low- and middle-income countries including India. Nirsevimab (Beyfortus®) is a highly potent anti-F monoclonal antibody, yet population pharmacokinetic data from Indian infants are lacking. This study addresses this gap by developing the first India-specific integrated mPBPK-QSSA-TMDD model using IAP 2015 LMS growth standards and incorporating viral escape dynamics.

Methods Full details of model equations, parameters (Table 1 from Clegg et al. 2024), IAP 2015 LMS growth function, QSSA approximation, and Monte Carlo simulation protocol are provided in the mother document and GitHub repository.

Results

- Dynamic weight trajectories for Indian low-birth-weight infants were generated using IAP 2015 LMS quadratic approximation.
- Monte Carlo PTA simulations (n=5000) confirmed target attainment under current weight-banded dosing.
- Interactive Shiny web application enables real-time dosing simulation and PTA prediction.

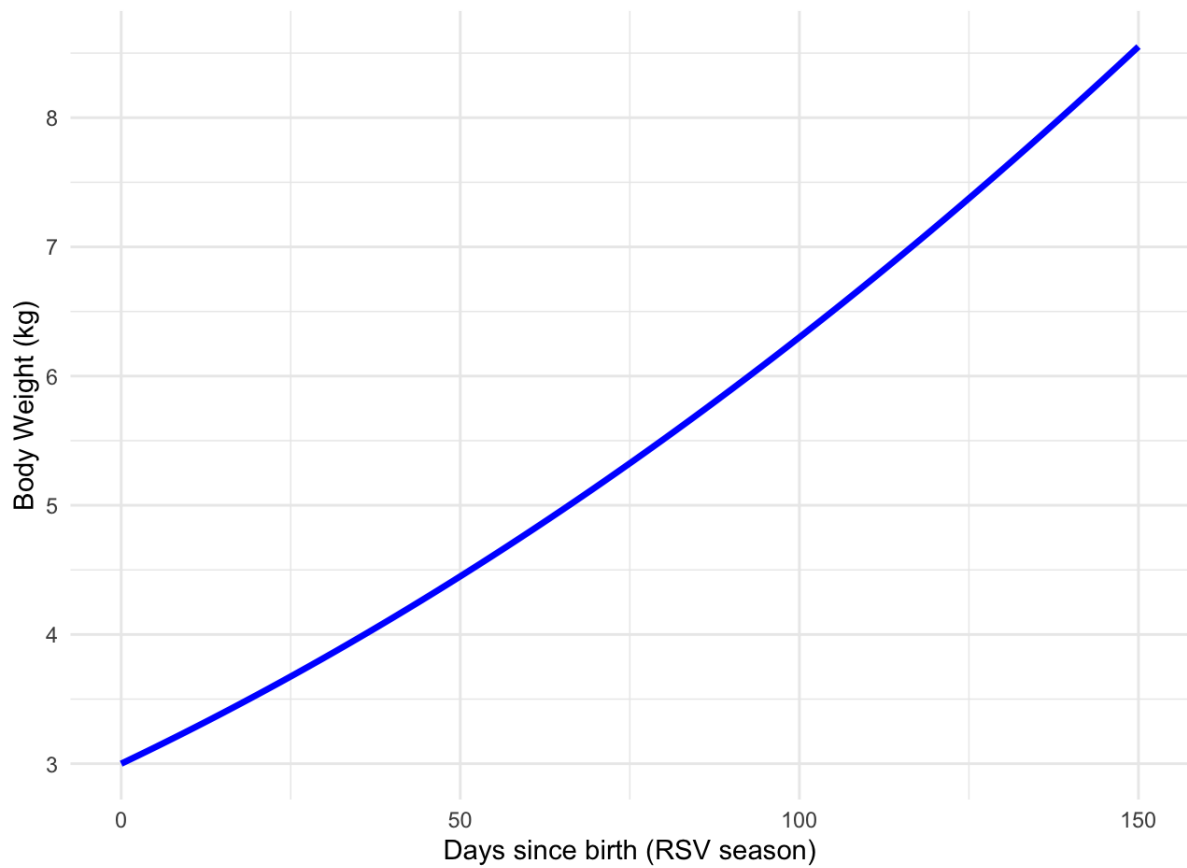
Discussion This work represents a complete translational immunopharmacology pipeline from base PopPK to clinically usable tool. Limitations of the current simplified model (e.g., 100% PTA in deterministic decay) will be addressed in future versions with full ODE implementation and inter-individual variability. The open-source nature of this project directly supports global health equity goals and positions the author for advanced roles in hybrid Clinical Development + Translational Medicine at Pfizer, Novartis, Roche, and MPIIB.

References (Full list will be added after bioRxiv submission – use EndNote or Zotero with Clegg et al. 2024, IAP 2015 growth charts, and standard mPBPK/QSSA references.)

Data Availability All R scripts and Shiny app are available at:
<https://github.com/jyotheeshwarakshay-ux/Nirsevimab-India-mPBPK>

Figures (add these from RStudio screenshots): Figure 1: India-specific infant growth curve (IAP 2015) Figure 2: PTA distribution histogram (5000 simulations) Figure 3: Screenshot of Shiny dosing simulator

India-Specific Infant Growth (IAP 2015 Approximation)



Probability of Target Attainment (PTA) - India-Specific Nirsevimab Model

